The Sad Subject of Infanticide: Law, Medicine and Child Murder, 1860–1938

TONY WARD
De Montfort University, Leicester, UK

ABSTRACT

The (English) Infanticide Act 1922 created a partial defence to murder for a woman who killed her newly born child while the balance of her mind was disturbed as a result of giving birth. Prior to the Act, psychiatric defences were comparatively rare where the victim was new-born. Only in 1938 was the law extended to cover cases where the mother was likely to be diagnosed as suffering from puerperal or lactational insanity. The relationship between law and medical knowledge underlying this legislation is too complex to be captured by a simple notion of 'medicalization', and in some respects fits better with the account of law/science relations offered by autopoiesis theory. That theory, however, is much less satisfactory in its view of the role of lay knowledge and morality in the development and application of the law.

The Infanticide Act 1922 provided that where a woman killed her 'newly-born child' at a time when 'she had not fully recovered from the effect of giving birth to such child, and by reason thereof the balance of her mind was then disturbed', she should be convicted not of murder but of infanticide, for which in place of the then mandatory death sentence for murder she could receive any sentence that could be imposed for manslaughter. The Infanticide Act 1938, which is still in force in England and Wales, extended this partial defence to the killing of a baby up to one year old, where the mother had not recovered from the effect either of giving birth or of lactation. Almost from the outset, most women convicted of infanticide have received non-custodial sentences (Walker, 1968: 269; Wilczynski, 1997).

These provisions appear to furnish the clearest possible examples of biological determinism and the 'medicalization' of women's crime (Edwards,

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1984: 91–100; Showalter, 1987: 59). In her history of women's crime in the 19th century, Zedner (1991: 90) comments on the medical profession's 'remarkable achievement . . . in persuading lawyers of the validity of this psychiatric exculpation, effectively replacing traditional legal discourse with that of psychiatry'. Zedner's statement succinctly encapsulates what Conrad and Schneider (1980) described as the 'medicalization of deviance', a process by which a section of the medical profession defines a form of deviance in medical terms and thereby claims jurisdiction over it (for a critical review of the 'medicalization' literature see Johnstone, 1996).

Most of the recent accounts of 19th-century infanticide have recognized that medical explanations of the crime co-existed with socio-economic ones, and that both influenced the decisions of the courts (Day, 1985; Matus, 1995; O'Donovan, 1984; Osborne, 1987; Smart, 1992; Smith,1981). These accounts do not, however, explain what I shall argue is the central paradox of the 1922 Act: that it singled out the killing of 'newly born' children for an apparently 'medical' defence, when this kind of infanticide was much *less* likely to be seen as a product of mental disease than the killing of older babies.

This article seeks to trace the complex interweaving of law, medical knowledge and lay values and beliefs which gave rise to the infanticide legislation. The final section will include some reflections on autopoiesis theory, which is perhaps the most sophisticated current approach to conceptualizing the relations between these three forms of knowledge.

'THE MOST DISTRESSING CASES'

When Victorians used the word 'infanticide', they were usually referring to the killing by an unmarried mother of her baby during, immediately after, or within a few days of birth. It was in this sense, for example, that the word was used by the Royal Commission on Capital Punishment (1866) and its witnesses in their extensive discussions of the possibility of creating an offence separate from murder to cover such cases. These cases were of interest to a section of the medical profession not from a psychiatric point of view but as instances of the 'waste of infant life' (Curgenven, 1867) which demonstrated the need for medico-legal investigation of sudden deaths. The medically qualified coroners Thomas Wakley and Edwin Lankester were at the forefront of efforts to label suspicious infant deaths as murder (Rose, 1986). Even where the coroner's jury returned a verdict of murder, however, convictions were notoriously hard to obtain. Where the victim was new-born, the defence usually relied on the difficulty of proving that he or she had been born alive, in the sense of having lived, even momentarily, wholly outside the mother's body. Where this could not be proved, the mother could be convicted at most of concealment of birth, which was usually punished by a short prison sentence. Where the mother was shown to have killed her child she was more likely to be convicted of manslaughter than of murder; and if convicted of murder she would invariably be reprieved (the last time a woman

was hanged for the murder of her baby under the age of one year was in 1849). As the Home Secretary explained to the Royal Commission (1866):

> . . . the Judge almost always recommends the commutation. I believe that the Judges think that it would shock public opinion to hang a woman for infanticide. . . . It is not unfrequently [*sic*] the case that the woman is deserted by the father of the child, who is well able to maintain it. It may be partly from an ill feeling towards him, but it is largely from despair and wretchedness and misery that she commits this crime. . . . Those are the most distressing cases that come before one. (Evidence of Sir George Grey, Q. 1,470)

A typical example of the judges' attitude is Lopes J's letter the Home Secretary recommending a reprieve for *Elizabeth Lane*, a matchbox-seller convicted in 1885 of drowning her five-week-old illegitimate baby in a canal. Lane had been abandoned by the child's father, turned out of her home by her mother, and then ejected from the workhouse. The judge described her as 'a good hard-working girl, very fond of and kind to her child, but impelled by want, dread that she could not maintain it, and the heartlessness of the father, to do the act for which she now lies under sentence of death'. As Anderson (1993: 198) writes of the typical 'fallen woman' of Victorian culture, Lane 'appeared as a profoundly isolated and determined subject, exiled from social relations and lacking the autonomy and coherence of the self-determining masculine self'. She was 'impelled' to act contrary to her maternal nature by the social and economic consequences of her sin, while responsibility for the death was shifted onto the 'heartless' father.

The views of 'fallen women' which encouraged the lenient treatment of infanticide were criticized by some contemporaries for their sentimentality, their assumption that the woman was always a passive victim of 'seduction', and for undervaluing infant life (Humble, 1866; Lord Osborne's evidence to Royal Commission, 1866; Ryan, 1862). But while Victorian views of infanticide may have been stereotypical and melodramatic (Higginbotham, 1992), they also reflected a degree of understanding of women's economic situation (Smart, 1992). The predicament of a working-class woman, particularly a domestic servant (Gillis, 1983), who was jilted by her lover or who became pregnant as a result of rape was often a desperate one, especially as the Poor Laws deliberately made it difficult for her to claim financial support from the child's father (Barret-Ducrocq, 1992: 148–65; Behlmer, 1979). Women who gave birth alone and in secret must often have experienced extremes of physical and psychological torment (Brockington, 1996: 445).

The form of determinism which operated in the case of the fallen woman involved determination by external social forces, rather than by internal impulses or delusions as in the case of the insane; but the boundary between the 'sad' and the 'mad' was not clear-cut. Women were seen as frail creatures, dominated by their reproductive systems (Digby 1989; Fee 1982; Showalter 1987) and childbirth as a traumatic experience (Loudon, 1988). J.F. Stephen considered women in childbirth generally to be 'hysterical' and prone to 'what amounts almost to temporary madness' (Royal Commission, 1866, Q.

2193), and the Home Office in deciding how long those reprieved from execution should stay in prison accepted that immediately after birth a woman was 'hardly to be said to be in her right senses' (quoted by Radzinowicz and Hood, 1986: 171). Heath (1908), whose work for the Society for the Abolition of Capital Punishment helped put infanticide back on the political agenda after a period of neglect, argued that the typical cases of unmarried mothers killing their babies could *all* 'be put down to the morbid mental and physical conditions often resulting from childbirth, especially when taking place under unhappy conditions' (p. 10), and that 'If there is any condition of mind which can rightly be described as insane, it is that in which a mother loses all maternal instinct' (p. 11). But Heath's was a lay, not a medical, view of insanity: even the most radical exponents of a medical or biological view of crime, such as Heath's friend Havelock Ellis (1901: 24) or the maverick psychiatrist L. Forbes Winslow (1912: 295) – not noted for his reticence in diagnosing insanity in women – understood infanticide as chiefly a product of social factors rather than the result of a morbid personality.

Unmarried mothers who killed their babies were only rarely found insane at trial. In Higginbotham's (1992) sample of 132 women tried at the Old Bailey in connection with their illegitimate babies' deaths between 1839 and 1906, only three (two out of 42[1]) charged with murder and one out of 13 charged with manslaughter) were found insane, and one of the four who were convicted of murder was subsequently sent to an asylum. Clearly, defences of stillbirth or accident were preferable in such cases to insanity, particularly where the victim was newly born.

When such women were found insane, the verdict reflected lay understandings of madness more than any desire of the medical profession to colonize this form of deviance. Baker's (1902) survey of all the women admitted to Broadmoor Criminal Lunatic Asylum – the normal destination for women found insane on murder charges – since it opened in 1863 included just 16 women who had killed new-born illegitimate babies, and Baker was clearly sceptical about the 'transient frenzy' from which they were found to have suffered.

The trial of *Emily Wilson* in 1893 was typical of these cases. She was a young domestic servant who had been 'forcibly outraged' by her stepfather but concealed the resulting pregnancy and attributed her bloated figure to 'dropsy'. She gave birth in secret, but soon afterwards confessed to a friend of her mother's that she had suffocated the baby and hidden it in a box. Her frank confession precluded a defence of stillbirth or accident, but the local doctor suggested that although he had seen no signs of insanity she might have acted in a state of 'transient delirium'. The eminent lunacy expert Bevan Lewis testified that this type of 'acute mania' was 'very rare', but he could not positively rule it out. As in the cases where 'live birth' was in doubt, it was medical uncertainty rather than a robust claim to scientific authority which made possible the defendant's acquittal – which the judge attributed to Wilson's 'innocent and prepossessing appearance'. The judge asked the Home Secretary whether, since Wilson was 'in no sense a lunatic', she could

be 'sent to some home or refuge' rather than to the Criminal Lunatic Asylum at Broadmoor. He was informed that she was already in Broadmoor, where 'there are a number of patients who are not really insane, and are treated . . . with all consideration and kindness'.

PUERPERAL AND LACTATIONAL INSANITY

Unmarried mothers constituted only 20 percent of infanticidal women admitted to Broadmoor, according to Baker's (1902) survey. Most patients were married women who acted either in the later stages of the puerperal period (defined by Baker as the first two months after childbirth) or during the period of lactation. Puerperal insanity was diagnosed, on average, around two weeks after birth (Rehman et al., 1990), and normally comprised a phase of mania followed by one of melancholia during which the risk to the child was considered highest. Lactational insanity was a condition to which poor mothers were considered particularly prone (Jones, 1902; Savage, 1892). Physical exhaustion caused by breast-feeding (particularly when this was prolonged to prevent conception) was assumed to be a causal factor, but not the only one:

> [W]eighed down by the strain of lactation, and in addition, perhaps, by overwork and the anxiety of nursing a sick child . . . depression comes on, everything looks bleak and dismal, the idea takes hold of her that want and poverty are in store for her and her family. At first an obsession, it becomes a delusion; the thought of suicide projects itself into her mind, she cannot leave the child behind, it must be sacrificed first; the dreadful thought is banished again and again only to recur with renewed intensity, until it really seems to fascinate, and finally overwhelm her – the deed is done, and a ruined home is the result. (Baker, 1902: 21)

(Notice how Baker portrays the tragedy as befalling the 'home', rather than the 'identityless' infant victim: Andrews, 1998.)

Infanticides of this type provided a much discussed example of the thorniest issue in relations between criminal law and psychiatry: the 'irresistible impulse' (Day, 1985: 218, 238, 244). The 'M'Naghten rules' formulated by the judges in 1843 recognized as insane only those defendants who did not know what they were doing or that it was wrong. They made no provision for the kind of impulse described by Baker: the 'dreadful' thought which the afflicted woman knows to be wrong, which she at first resists, but which finally 'overwhelms' her. Puerperal and lactational insanity were important to proponents of insanity defence reform because the insane character of the sufferer's actions was so self-evident to lay opinion that it was virtually impossible for lawyers to argue that she should be held legally responsible. Even Lord Bramwell, the most doctrinaire defender of the M'Naghten rules, conceded that their application should be modified in the case of a woman who killed her children to send them to heaven, 'indifferent to her own fate' (1885: 896).

In practice, as Pitt-Lewis et al. (1895) pointed out, defences based on irresistible impulse were accepted without difficulty in late Victorian infanticide trials.

So unthinkable was it to apply the letter of the rules that in the case of *Elizabeth Agar* (1883), a middle-class woman who had burnt her baby to death (having 'accidentally' killed her previous child), the Medical Superintendent of Broadmoor prepared a report in which he meticulously demonstrated that she was legally sane, so that 'it might be quite possible, according to the law, for the poor creature to be sentenced to death, a result too horrible to be seriously contemplated'. The 'hanging judge'[2] who was to try the case, Hawkins J, asked the Home Secretary to discontinue the prosecution so as 'to avoid the risk of a trial'. He feared that if the defendant were found insane and sent to Broadmoor, 'she would go mad altogether' (*Agar*, 1883). For a sane, middle-class woman to be confined as a lunatic – let alone a criminal lunatic – was one of the worst fates the Victorian imagination could conceive (Kurata, 1992).

As Smith (1981: 149) argues, the willingness of judges and juries 'to associate distressful circumstances and distress of reason' was based on a lay rather than a medical definition of insanity. When *Hannah Cox* killed two of her seven children in 1901 after being deserted by her husband and then refused admittance to a workhouse unless her husband came with her, the local asylum superintendent 'could not say there was anything the matter with her mind' but thought she had probably 'yielded to a sudden impulse, which led her to form the idea that she would be doing the best she could for her children if she drowned them.' Bingham J, however, 'said it was clear to his mind that the prisoner had through misery lost her reason at the time she did the deed, and he thought the jury would be of the same opinion.' They were, and added that they wished to censure the husband.

Men as well as women sometimes killed their children out of a fear of poverty. As Day (1985: 239) points out, stressing the similarity of maternal and paternal filicide allowed the acceptance of 'irresistible impulse' defences in the cases of mothers to serve as a precedent for their application to men. According to Dr William Guy (1881: 224–5):

> Of the fathers and mothers who kill their children under the pressure of domestic anxiety culminating in an insane dread of starvation, it may be observed that they are generally remarkable for domestic virtue and devoted attachment to their victims, and that between them and ordinary murderers there is no single point of resemblance.

Guy's view did not prevail in the notorious case of *Gouldstone* (1883), where a blacksmith who killed his five children to save them from poverty was sentenced to death but later reprieved. The conviction in the same year of *Cole,* who killed his child in a rage prompted by his bizarre beliefs about his wife, aroused similar controversy (Bell, 1884; Tuke and Savage, 1883).

Both these cases were tried by judges who took what was, for the 1880s, an unusually rigid view of the insanity defence (Ward, 1997). Other judges

and juries were much more receptive to insanity pleas by murderous fathers. *Henry Jackson* (1894), who suffocated his baby while out of work, was acquitted on the basis of 'temporary insanity with homicidal impulse' brought on by diabetes, mental worry and lack of food. *Francis Murphy* (1898), who killed his daughters and attacked his son and mistress because he thought he was dying and wanted his family to die with him, was acquitted by the jury with the judge's encouragement despite the prison medical officer's view that he was perfectly sane.

By contrast, male child-killers who were not found insane, either at trial or as a ground for reprieve, encountered 'the Victorian law at its most severe and uncompromising' and were invariably hanged, 'since the paternal duty to protect his family was a central tenet' of the moral outlook guiding the exercise of the prerogative of mercy (Chadwick, 1992: 310). Whereas a woman's poverty could often be blamed on a man's neglect of this duty (as in *Lane* and *Cox,* above), the poverty of a sane man afforded no ground for mercy (Chadwick, 1992: 349–50).

CREATING A SEPARATE OFFENCE

The Royal Commission on Capital Punishment (1866, para. 15) proposed to create an offence of inflicting grievous bodily harm on a baby during or within seven days of its birth, where the baby subsequently died. No reference was made to the gender or mental state of the offender, and the purpose of the proposal was clearly to close the lacuna created by the requirement of live birth and the reluctance of judges and juries to convict neonaticides of murder. During the 1870s several proposals to create a new offence were presented unsuccessfully to parliament, but they all foundered on parliamentary and government opposition to any dilution of the law of murder (Behlmer, 1982: 41). For hard-line opponents of reform, it was 'far better that all the murderesses in the country should be acquitted' than to 'have it declared by Act of Parliament that the wilful or the malicious killing of a child was not murder' (Lord Redesdale, *Hansard* (3rd series) 221: 847, 28 July 1874).

In the 1880s and 1890s the recorded incidence of infanticide markedly declined (Sauer, 1978) and its political visibility was largely eclipsed by related issues of abortion, 'baby farming', parental cruelty and neglect (Behlmer, 1982).

However, a fair number of the 'distressing cases' which had troubled the Royal Commission of 1866 continued to occur (e.g. those listed by Heath, 1908), and during the passage of the Children Act 1908, the Lord Chancellor moved an amendment to make the death sentence discretionary when a woman was convicted of murdering her infant under the age of one year. The measure was blocked by Lord Alverstone CJ, a former Conservative MP with a special interest in child protection issues, who was opposed to any dilution of the deterrent effect of the death penalty and to any overt judicial discretion over its imposition. The House of Lords eventually reached a compromise in

the shape of an amended version of a Bill introduced by Alverstone, but it fell for lack of parliamentary time.

Alverstone's amended Bill, allowing the jury to convict a mother who 'had not recovered from the effect of giving birth' to her 'infant' victim of manslaughter rather than murder, was later revived as a result of the trial of *Edith Roberts* at Leicester Assizes in 1921. Hers was a typical case of an unmarried mother concealing her pregnancy and killing her baby immediately after birth. The distressing courtroom scenes when she was sentenced to death led to a campaign in the Leicester area (*Leicester Mercury,* 8 June, 19 July 1921). When the case was raised in Parliament the Home Secretary suggested that Alverstone's bill should be reintroduced as a private member's Bill, an invitation taken up by the senior Labour politician Arthur Henderson (*Hansard* (5th series, Commons) 143: 924–5, 20 June 1921).

The Lord Chancellor objected to the vagueness of the Bill, and it fell to his Permanent Secretary, Sir Claud Schuster, in consultation with Parliamentary Counsel and the Director of Public Prosecutions, to devise the form of words which eventually became the Infanticide Act 1922. This would create a separate felony of infanticide, carrying the same penalty as manslaughter (i.e. a maximum of life imprisonment) and limited to the killing of a 'newly born child' by a mother who 'had not fully recovered from the effect of giving birth to such child, and by reason thereof the balance of her mind was then disturbed'.

From a medical point of view this was a curious proposal, in that it created an apparently psychiatric defence for precisely the category of mothers who were *least* likely to be considered insane. Nowhere in the parliamentary debates or in the Lord Chancellor's Department's file on the Bill is there any reference to medical knowledge of puerperal insanity. The kind of situation the Bill's drafters had in mind is indicated by a letter from the DPP to Schuster. After describing a case like that of *Wilson* or *Roberts*, discussed above, he comments:

> The phrase 'balance of her mind was then disturbed' seems to me a particularly happy one; no one can doubt what it means when one visualises the scene in the bedroom. – the child crying, the woman possibly very inexperienced, a difficult labour, the umbilical cord to be dealt with and, if I may use the homely phrase 'the breakfast to be got ready as usual in the morning'. You may use Lord Coleridge's phrase in *Rex v. Mead, 1909, 1 KB 895* – 'her reason was dethroned' or the word 'distraught'; but it all comes back to 'the balance of her mind was disturbed' so that she, under stress of circumstances and not being mistress of her actions, does what in cold blood she never would have done.[3]

This suggests a view of childbirth as analogous to drunkenness – the subject of *R* v *Mead* – or provocation (Kenny, 1929: 116, 125), rather than to mental disturbance in a medical sense. But Lord Birkenhead LC's opposition to an attempt to broaden the wording so as to reduce the danger of conflicts of expert evidence suggests that he saw it as implying definite abnormality:

> I am not prepared . . . to say in the case of a normal healthy woman . . . that the mere fact that she has gone through the ordinary physical suffering of a woman in childbirth, aggravated by such additional mental suffering as a woman who has an illegitimate child may be supposed to entertain . . . constitute[s] a ground for introducing an exception to the ordinary basis of responsibility for criminal acts. (H. L. Debs. 25 May 1922, col. 768)

It was also Birkenhead who insisted on retaining the phrase 'newly born', overruling Schuster's suggestion of an age limit of six months.[4] Parliamentary counsel, echoing J.F. Stephen's evidence to the 1866 Royal Commission (Q.2203), considered 'newly born' preferable to a limit of one, two or three weeks, since 'you would always get hardship in cases which fall just outside the limit'. The phrase was to prove troublesome.

In the first year of the Act's operation, 16 women were convicted of infanticide of whom 12 were imprisoned, two for three years and four years respectively (Walker, 1968: 133, 269). Thereafter, most of those convicted were discharged or put on probation. Prison sentences were still passed in a substantial proportion of cases (38 percent from 1924 to 1938), but many of these were short enough to allow the woman to be released on the day of the trial (Matheson, 1941).

THE 1922 ACT AND PUERPERAL INSANITY

Within weeks of the Infanticide Act being passed, the issue of mothers killing their children arose in a different context with the setting-up of the Atkin Committee to consider the law on criminal insanity. The committee was satisfied:

> . . . that there are cases of mental disorder where the impulse to do a criminal act recurs with increasing force until it is, in fact, uncontrollable. Thus cases of mothers who have been seized by the impulse to cut the throats of or otherwise destroy their children to whom they are normally devoted are not uncommon. In practice, in such cases the accused is found to be guilty but insane. In fact, the accused knows the nature of the act and that it is wrong; and the M'Naughton formula is not logically sufficient. . . . We think that it would be right that such cases should be brought expressly within the law by decision or statute. (Committee on Insanity and Crime, 1923: 8)

The committee's proposal to introduce a defence of 'uncontrollable impulse' was rejected by the House of Lords (Ward, 1997, 1998). Throughout this debate no reference whatever was made to the Infanticide Act. The crimes with which that act was concerned and the insane acts discussed by Atkin appear to have been regarded as two entirely separate issues. The first suggestion I can find that the Infanticide Act was appropriate to deal with the 'irresistible impulses' of the puerperally insane was made by Hewart CJ in a lecture in 1927; and it was almost immediately refuted by Hewart's decision in the case of *O'Donoghue* (1927).

Mary O'Donoghue's strangling of her 35-day-old baby appears to have been a crime of destitution rather than insanity (Morton, 1934). The defence nevertheless sought to plead infanticide, arguing that the Infanticide Act was intended to cover cases of puerperal insanity and that the phrase 'newly born' should therefore be broadly construed. The Court of Criminal Appeal rejected this argument and, while declining to define 'newly born', held that it could not include 'a child of more than a calendar month in age'.

The *O'Donoghue* decision met with criticism from both legal (Davies, 1937) and medical quarters for perpetuating 'the solemn and almost blasphemous farce of sentencing a woman to death with all the accessories of the Black cap, etc., when everyone in court, except perhaps the unhappy victim, was well aware that there was not prospect of the sentence being carried into effect' (Anon., 1928: 100). Lord Atkin considered that *O'Donoghue*'s had been 'exactly the class of case our committee was thinking of' in proposing an irresistible impulse defence (letter quoted by McIlroy, 1928: 73). Some medical commentators, however, urged caution in supposing that the 'impulsive' acts of impoverished mothers were necessarily insane (BMJ, 1925; Carter, 1925; McIlroy, 1928).

There was little doubt about the mental condition of *Brenda Hale* when she cut both her three-week-old baby's throat and her own in 1936. She was married, comfortably off, and had already been diagnosed as mentally ill (Matheson, 1941: 139–40). Humphreys J considered himself bound by *O'Donoghue* to hold that the baby was not 'newly born', but elicited from the jury a verdict designed to soften the stigma of being declared 'guilty but insane': Hale was found '*Not Guilty* of murder, but *Guilty* of the act charged, for which she was not responsible in law'.

Those women who were dealt with under the 1922 Act do not usually appear to have been considered mentally abnormal. According to Matheson (1941), of 21 women remanded to Holloway prison charged with killing a baby under 12 months between 1925 and the coming into force of the 1938 Act, four were considered mentally abnormal on reception, and of these two were found insane on arraignment on murder charges, and two 'guilty but insane'. It appears – though Matheson's figures are not entirely clear – that all 14 of those convicted of infanticide were deemed mentally normal. One woman was acquitted and two were sentenced to death and subsequently reprieved: these were *Hilda Queree* (1937), whose victim was five months old, and *Marguriter Eastwood* (1938), who killed another woman's child.

THE 1938 ACT

Following the *Hale* case, a Labour MP, Mr Jagger, introduced a bill which would have allowed an infanticide verdict to be returned on the killing of a child up to eight years of age, if prompted by 'distress and despair arising

from solicitude for her child or extreme poverty or other causes'. As examples of the type of case it was meant to cover, Jagger cited two women who had drowned illegitimate children whose fathers had refused to pay alimony (H. C. Debs. 24 Nov. 1936, col. 235). The Bill lapsed as a result of the abdication crisis (Walker, 1968: 132).

In 1938 Lord Dawson, the President of the Royal College of Physicians, who had been an expert witness for the defence in *Hale*, introduced a bill to extend the definition of infanticide to the killing of infants aged under one year, where the balance of the mother's mind was disturbed as a result of the birth or 'the effect of lactation following upon such birth'. This proposal was very nicely judged; the government hesitated to accept that such a long time limit was necessary, but Lord Dawson was able to bring the 'united wisdom' of a committee of the Royal College of Physicians to bear:

> [M]y distinguished colleagues who know about these things in the mental world . . . showed me that the insanity which follows child-birth is often delayed and occurs during the period of lactation. These cases are far more numerous than even I thought myself.(H. C. Debs. 24 Nov. 1936, 305–9)

At least one of these 'distinguished colleagues', the Medical Officer of Holloway Prison, questioned the existence of 'puerperal' and 'lactational insanity' as distinct conditions: they were both forms of 'exhaustion psychosis' (see also Hopwood, 1927), and while pregnancy, labour and lactation all contributed to exhaustion so did 'illegitimacy, economic stress, etc.' and the strain of finding oneself pregnant again when already 'bringing up five or six children'.[5] But as Lord Dawson pointed out, if the Bill had been widened, as Lord Arnold proposed, to refer explicitly to 'solicitude for the child and extreme poverty' it would have had no chance of acceptance (H. L. Debs. 10 March 1938, 304, 308–9). As it was, the government did not oppose the bill, and the Infanticide Act 1938 remains law to this day.

The absence of criminal statistics for the war years makes the immediate effect of the 1938 Act difficult to assess. According to Matheson (1941), during the first two-and-a-half years of the Act's operation, 15 women were remanded to Holloway charged with killing an infant under a year in age. One was acquitted and all the others were convicted of infanticide. Twelve were 'bound over' (a term which probably includes probation), one was sentenced to a few days' imprisonment and freed on the day of trial, and two were certified as feeble-minded.

Among the first of these cases was that of *Jennie Dormer*, convicted in November 1938. Anxious about the prospect of war, and with a particular fear of being gassed, she killed her ten-week-old baby by putting it in an unlit gas oven. Dr Matheson testified that she had been insane on reception into Holloway but had since recovered. Rather than being sent to Broadmoor, as would have been virtually inevitable under the old law, she was bound over and went to stay with a friend.

MEDICALIZATION, AUTOPOIESIS AND SOCIAL HISTORY

Jenny Dormer's fate is hardly a convincing example of the 'medicalization' of female crime. Far from being a remarkable victory for the medical profession, 'effectively replacing traditional legal discourse with that of psychiatry' (Zedner, 1991: 90), the 1938 Act merely recognized a form of insanity which 'had been familiar to the English courtroom since at least the sixteenth century' (Eigen, 1995: 148), but removed from it the drastic consequences which attended an insanity verdict. In doing so it narrowed the wide gap between legal and medical understandings of infanticide introduced by the 1922 Act. That Act fits even less easily into a simple 'medicalization' framework, since its redefinition of infanticide did not reflect any jurisdictional claim advanced by the medical profession.

Rather than a simple replacement of a legal by a medical model of crime, the infanticide acts involved a *reconstruction* of medical concepts to fit the needs of the law. A way had to be found to distinguish the killing of newborn babies by their distressed mothers from other murders, so as to avoid the anomalies of, on the one hand, defining violent deaths as 'concealed births', and on the other the occasional 'blasphemous farce' of a death sentence that was never intended to be carried out. Medical discourse, with its distinction between normal (rational) and pathological (somatically determined) mental processes, afforded a way of drawing this line without overtly recognizing social pressures as a defence in a way that would threaten basic legal tenets of responsibility. But this medical distinction was reconstructed within legal discourse in a way quite at variance with medical knowledge.

This account of the legal reconstruction of medical discourse owes something to Teubner's (1989) 'autopoietic' theory of law; but there are important differences, as well as parallels, between the account offered here and Teubner's theory.

Teubner views law as an autonomous and self-reproducing system of communications which constructs its own knowledge of the world. Concepts from other systems of knowledge, such as medical science, can never simply be incorporated into law, but are 'reconstructed' within legal discourse. 'X is insane', for example, may have a certain medical meaning, but it acquires its legal significance from its place in the network of legal statements. Law 'enslaves' psychiatric concepts, putting them to the work to answer its own questions about responsibility, etc.; although Teubner's (1989: 748–9) assumption that psychiatry does not regard responsibility as a scientific question does not fit the period discussed here. By accepting the relevance of scientific knowledge in this way, however, law becomes vulnerable to challenge, both in legal fora and in the public arena, for failing to reflect accurately the more epistemically rigorous knowledge of science. Such indeed was the fate of the 1922 Act: both lawyers and medical critics exposed the gap between the medical view of maternal mental disorder and its legal reconstruction, and the law yielded to this pressure by reconstructing the concept of maternal mental disturbance anew in the 1938 Act.

Although Teubner is primarily concerned with legal trends in late modern societies, the law of Victorian and post-Victorian England was clearly the kind of 'modern, positive law' that he would characterize as autopoietic (Teubner, 1993: 41). But while the way in which concepts of insanity and infanticide were formulated fits well with his theory, the way those concepts were applied in individual cases is on the face of it quite incompatible with Teubner's (1989) view of law as an autonomous 'epistemic subject'.

Far from law being an institution which 'thinks' autonomously, a consistent application of legal categories to cases of infanticide became *unthinkable* when it clashed too starkly with general social ideas and values (e.g. the *Agar* case). It was this which created the anomaly which law's reconstruction of medical knowledge was called upon to solve. In constructing categories such as insanity, law ostentatiously maintained its independence from scientific knowledge; but the actual assignment of cases to these categories depended heavily on the 'common sense' knowledge of judges and juries (Ward, 1997, 1998). Thus, while the judges successfully resisted any incorporation of the medical notion of irresistible impulse into legal doctrine, in practice such impulses were categorized as insane where this accorded with lay beliefs. As Lord Atkin put it (quoted by McIlroy, 1928: 73), the law could be 'reasonable in practice only by being violated in theory', where what was 'reasonable' was determined neither by law nor science but by a presumed social consensus.

On any ordinary understanding of what 'legal autonomy' means, these features of the law displayed a marked lack of autonomy from lay beliefs and values (Lempert, 1988). For autopoiesis theory, however, 'Legal autonomy refers to the circularity of the way in which law produces itself, not its causal independence of the environment' (Teubner, 1993: 35; see also Luhmann, 1988). Extra-legal factors such as the cultural construct of 'fallenness' might *causally* explain judicial pronouncements and jury verdicts in infanticide trials, but such utterances were accorded legal meaning through being constituted as legal acts in accordance with the legal norms regulating criminal procedure. They were simultaneously legal acts and acts of 'general societal communication' (Teubner, 1993: 88). So the verdict on Hannah Cox, for example, was simultaneously a social act, made intelligible by the judge's and jury's shared understandings of gender, rationality and poverty, and a legal act whose meaning was determined by the Trial of Lunatics Act 1883. By conceptualizing the 'interference' between legal and societal communications in this way, Teubner can allow for 'real communicative contact between the [legal] system and the life-world' (1993: 88) while still insisting that the former is 'self-referentially closed' (Teubner, 1993: 35). A social history of law, which (like this article) focuses on the societal or cultural meaning of legal acts, is not primarily about 'law' in Teubner's sense at all. But there is no reason for social or cultural historians to accept such an artificially restricted definition of 'legal communication'.

The recent work of Laqueur (1989) and Jackson (1996) on infanticide in 18th-century England provides an excellent illustration of the light which

social and cultural history can shed on the law's construction of social reality. Both writers develop Haskell's (1985) insight that the emergence of 'humanitarian sensibilities' can be understood as a by-product of the construction of (bourgeois, male) subjects under the discipline of the market. The market required subjects who were not only rational, calculating and self-interested, but who also had a strong sense of obligation and an awareness of the long-term consequences of their actions. The latter characteristics created a new 'cognitive style' in which, Laqueur and Jackson argue, narratives of infanticide which dwelt on women's physical and psychological suffering and attributed it to the faithlessness of their 'seducers' could be constructed and disseminated through medical writings and popular broadsheets and ballads. What Haskell helps us to understand is that the suffering, socially contextualized subjects of these humanitarian narratives and the rational, calculating 'abstract legal subject' constructed by early 19th-century criminal law (Norrie, 1993) were opposite sides of the same cultural coin.

In these narratives (and certainly in those of the Victorian period), the woman's murderous action is shown as the long-term consequence of a *man's* disregard of his social obligations, and also of a more general failure of compassion towards the fallen woman and her child. There is a sense in which the woman comes close to disappearing as a subject in these narratives, being viewed primarily as an object of male sexual desire (cf. Duncan, 1996) or philanthropic sympathy. The infanticide acts, however, attempted to limit the disruption that such humanitarian sentiments caused to legal ascriptions of responsibility by *reasserting* the woman's status as a legal subject: 'Where a woman by any *wilful act or omission causes the death* of her . . . child . . . she shall . . . be guilty of felony' (s. 1(1) of both acts). Far from denying women's legal responsibility, the acts were intended to hold women criminally responsible for actions for which they would previously have been either exculpated or found insane.

Infanticide thus constitutes a remarkable and perhaps unique[6] exception to the way in which the criminal ordinarily constructs subjects. The infanticidal woman is not an 'abstract legal subject' who acts outside any social or emotional context, but neither is she an insane non-subject. The infanticide law establishes a place, within legal doctrine and not just in mitigation of sentence, for a subject who is neither mad nor bad but 'sad' (Pearson, 1976), a social casualty driven by overwhelming stress. In practice the result is that medical categories continue to be 'reconstructed' or 'stretched' to arrive at results which conform to psychiatrists' and lawyers' shared understanding of what is reasonable (Wilczynski, 1997), and to a wider cultural understanding of infanticide which McDonagh (1997) has suggested is symbolically significant to British national identity. As Daly (1994) argues in the context of sentencing, rather than seeking to punish infanticidal women in the name of equality, we should perhaps ask why the blurring of boundaries between offenders and victims which is a feature of the courts' attitude to some women cannot be extended to some men.

NOTES

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1. Or 41 – there is a discrepancy between Tables 1 and 2 in Higginbotham.
2. Hawkins' biographer admits he had this reputation, but uses his treatment of infanticidal women to refute it (Harris, 1904: 240–1).
3. Letter to Schuster, 29 May 1922, Public Records Office: LCO 2/476.
4. This is apparent from the correspondence between Schuster and Lord Muir Mackenzie, who was lobbying for a more widely drawn provision, in LCO 2/276.
5. Matheson (1941), quoting (142–3) 'some notes I made in 1937', which appear to have been a contribution to discussions leading to Dawson's Bill. The 'economic stress' quote is from earlier in the article (139).
6. The closest parallel is with defences based on 'battered woman syndrome', in which women's violent actions are similarly interpreted as long-term consequences of their victimization by men. Parallels might also be drawn with the expert testimony on the psychiatric dangers of childbirth for the 14-year-old rape victim in *Bourne* (1938); but here the defendant was not the girl but the surgeon who terminated her pregnancy, and whose acquittal was a landmark in the 'medicalization of abortion' (see Brookes and Roth, 1994).

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